

Worked example — country action plan

Reference · Results Communication · ~10 min to read

What this handout is

A fully-filled example of a country action plan, built around one finding — **low rates of haemoglobin measurement during antenatal care**. Use it as a reference while you fill in your own action plan worksheet. Notice how each column gets a specific, concrete answer — not a vague one.

The finding — Only **42%** of lower-level health facilities systematically record haemoglobin (Hb) measurement during antenatal care visits. Risk: undetected maternal anaemia.

I. Actions and decisions

Column	Worked answer
Problem identified	Low rate of Hb measurement at ANC: only 42% of lower-level facilities systematically record Hb. ⚠️ Increased risk of undetected maternal anaemia.
Decisions and actions	1. Run targeted training on Hb-at-ANC protocols. 2. Equip priority facilities with hemocues + reagents. 3. Step up community education on Hb checks.
Constraints	Low availability of hemocues + reagents · Incomplete ANC registers · Providers under-trained · Low community demand.
Solutions	Include Hb measurement in the essential-inputs list · Set up a training + community-sensitisation plan.

II. Dissemination and use

Column	Worked answer
End user	National: RMNCAH-N coordination platform, Directorate of Reproductive Health. Regional/district: district health managers, supervision teams.
Message objective	Make decision-makers aware of the gap in Hb-at-ANC coverage · Mobilise resources for inputs and training.
Key messages	● "Only 42% of facilities measure Hb at ANC — thousands of anaemic pregnant women go undetected." · 📊 Trends and district-level analysis · 📦 Documented input shortfall · 📋 Recommend: make Hb recording mandatory in the HMIS by Q3 2026.
Dissemination channels	Quarterly RMNCAH-N meetings · District performance reviews · Monthly bulletins (PDF/email) · District debrief workshops · Parliament / donor reports.

Timeline

When	Milestone
May 2026	RMNCAH-N meeting (25 May) – present findings
June 2026	Debrief priority districts · launch training plan
September 2026	Mid-term review
December 2026	Annual review and report

Next steps + owners

Step	Owner
Add to the 25 May RMNCAH-N meeting agenda	RMNCAH-N Director
Finalise and validate the data presentation	Project lead
Draft the hemocue-supply plan	Logistics unit
Design the training module	MoH Training Division

Monitoring + evaluation indicators

- % of quarterly reports presented in relevant meetings
- Number of facilities equipped with hemocues
- Number of providers trained on the Hb protocol
- **% of facilities recording Hb at ANC (target: ≥ 80% by March 2027)**

What to notice in this example

Three things make this plan actionable, not aspirational:

1. **One specific person owns each step**, not "the ministry" or "the unit".
2. **Every action is checkable** — "train 50 providers", "equip 30 facilities" — you can tell next quarter whether it happened or didn't.
3. **The timeline is real dates tied to existing meetings** — 25 May, June district debriefs, mid-term in September. No "as soon as possible".

Use this as a model when you fill in your own action plan. If your worksheet doesn't have a specific person, a checkable action, and a real date for each row — it isn't ready yet.

What's next

Take this example into the **country action plan** activity. Fill in your own plan using your country's findings. Trade with another country team for peer review — ask them: would they be able to check, three months from now, whether each row got done?